

Surgery for advanced melanoma

Surgery can sometimes be used to help control advanced melanoma and relieve symptoms.

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About surgery for advanced melanoma

Surgery for [advanced melanoma](#) is not commonly done. But some people might have surgery if:

- there is only 1 tumour
- there are a few tumours, but they are all in 1 area
- the tumours are not too big
- a tumour is causing difficult symptoms.

This may be when the melanoma has spread to:

- areas of skin or lymph nodes far away from where the melanoma started
- the brain
- the liver
- the lung.

The type of operation you have depends on where the melanoma has spread. Your operation will be done by a specialist surgeon. The type of surgeon will depend on the part of the body that is affected.

Surgery to remove secondary melanoma in the brain, liver or lung is a big operation. The surgeon will check first to make sure it is suitable for you. They will talk to you about the possible benefits and risks of the surgery before you decide.

You usually need scans to check the melanoma has not spread anywhere else. You will also have other tests to make sure you are well enough to cope with the operation.

Before your operation, the surgeon and nurse will talk it over with you. It is important that you understand what it may involve. They will explain what to expect and how to [prepare for your surgery](#).

You may have other treatments before or after your operation.

Melanoma that has spread to the skin

You may have surgery to remove melanoma that has spread to areas of skin far from the primary melanoma. Melanoma that spreads to the skin can cause new lumps. Sometimes these lumps bleed or cause discomfort. They can be removed under local or general anaesthetic, depending on the amount of skin being removed.

You may have the operation as a day case. Or you may need to stay in hospital overnight. Some people need skin taken from another part of the body to replace the skin that is removed. This is called a [skin graft](#). Your surgeon or specialist nurse will explain more about this.

Your stitches will be removed about a week after surgery. If you have dissolvable stitches, they will not need to be removed. Your wound may need to be cleaned and dressed until it has healed. The hospital nurses can show you how to do this. Or they can arrange for a district nurse to do it for you at home.

Melanoma that has spread to the lymph nodes

Melanoma can spread to [lymph nodes](#) far away from the primary melanoma. The lymph nodes may become enlarged or feel hard. If this is uncomfortable or painful, you may have an operation to remove them.

You will have the operation under general anaesthetic. You may be in hospital for a few days. The type of operation you have depends on where the lymph nodes are. Your surgeon or specialist nurse will tell you more about what to expect.

After the operation

You may have a small tube (drain) to remove any fluid that builds up around your wound. The drain is connected to a small bottle. Your nurse will remove the drain when most of the extra fluid has drained away. This is usually within a few days.

Some people go home with the drain in place. It can be removed by a practice nurse at your GP surgery or a district nurse at home.

The wound will be covered with a dressing. Your stitches or staples will be removed 7 to 10 days after the operation. If you have dissolvable stitches, they will not need to be removed. The hospital nurses can show you how to take care of your wound. Or they can arrange for a district nurse to do it for you at home.

If your lymph nodes have been removed, you may develop swelling in an arm, leg or other part of the body. This will depend on where in the body the lymph nodes were removed. This is called lymphoedema.

We have more information about how to [reduce your risk of lymphoedema](#).

Melanoma that has spread to the brain

If melanoma has spread only to the brain, your doctor may talk to you about having surgery.

You may have surgery in combination with [immunotherapy](#) or [targeted therapy](#). A specialist surgeon called a neurosurgeon will explain whether an operation is possible.

Your neurosurgeon and specialist nurse will tell you what to expect before and after your operation. You will probably be in hospital for at least a week. You usually have [steroid drugs](#) to help reduce any swelling around the tumour and improve your symptoms. You usually have these before your operation and for a few weeks afterwards. If your surgeon cannot remove all the tumour, they will remove as much as they can.

You may have a type of radiotherapy called [stereotactic radiosurgery](#) after surgery to the brain.

Melanoma that has spread to the liver or lungs

Surgery to remove secondary melanoma from the lungs or liver is a big operation.

You may be in hospital for up to a week. But it may take several weeks or longer to recover. You may have surgery after you have finished [immunotherapy](#) or [targeted treatments](#). These may help to shrink the cancer.

An operation to remove secondary tumours in the liver is called a [liver resection](#). It takes several hours and is done in a specialist liver unit. After surgery, the remaining liver can grow bigger and work as it did before.

Surgery to the lung will depend on how much of the lung is affected. It may be a small section of the lung, or a larger area. Removing part of the lung will not affect your breathing.

About our information

References



Below is a sample of the sources used in our advanced melanoma information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Keilholz U, Ascierto PA, Dummer R, et al. ESMO consensus conference recommendations on the management of metastatic melanoma: under the auspices of the ESMO Guidelines Committee. *Annals of Oncology*. 2020. 31 (11): 1435-1448 [accessed May 2022].

Peach H, Board R, Cook M, et al. Current role of sentinel lymph node biopsy in the management of cutaneous melanoma: A UK consensus statement. *Journal of Plastic, Reconstructive & Aesthetic Surgery*. 2020; 73, 1, 36-42 [accessed May 2022].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Professor Samra Turajlic, Consultant Medical Oncologist.

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Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 October 2022 | Next review: 01 October 2025

⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help